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Maplight Therapeutics

Equity Research
March 15, 2026

Miami Dinner: Significant Upside Pot'l on Phase II Schizophrenia Data in Q3:26

Our discussion focused on ML-007C-MA's (oral M1/M4) differentiation vs BMY's Cobenfy (approved M1/M4) in **schizophrenia (SCZ)** and pivotal Phase II data expectations in Q3:26 (+100% stock move). ML-007 also has pivotal Phase II **Alzheimer's psychosis (ADP)** data in H2:27 (another +100% move). Meanwhile, ML-004 (oral 5-HT1B/1D) has Phase II **autism** data in Q3:26. Net-net, $1B+ peak sales x low 2-3x multiple = $2-3B of value per program (vs $950M + cap).

***** See 20+ pages inside for nuanced details *****

In Q3:26, ML-007C-MA could produce differentiated 5-week Phase II SCZ data. (1) ZEPHYR (N=300) randomizes patients 1:1:1 to **210/3mg BID**, **300/6mg QD** or pbo, with Week 5 PANSS as the primary endpoint. **(2)** ML-007C-MA is a fixed-dose combo of ML-007 (strong M1/M4 agonist) + fesoterodine (anticholinergic). '007 drives efficacy, while fesoterodine minimizes M1/M4 side effects (e.g., nausea, vomiting). **(3)** Each arm is >90% powered to detect an effect size (over placebo) at a discount to BMY's xanomeline (M1/M4)+trospium (approved as Cobenfy in 2024), which showed absolute PANSS benefits of -20-21 (vs -11-12 pbo) in Phase III (EMERGENT-2/3), or a **-8-10 pbo-adj delta.**

In short, mgmt's base case for ML-007C-MA is (1) competitive efficacy plus (2) better safety/tolerability (at least for BID) and (3) better ease-of-use. Simply having a QD with similar efficacy/safety to Cobenfy (BID) is upside. In any case, we think the 3 attributes are interconnected, since improving tolerability/compliance can lead to fewer discontinuations, enhancing efficacy. **(a)** On **efficacy**, MPLT's stronger M1/M4 agonism increases the likelihood of benefitting all SCZ domains, including negative symptoms. That said, Cobenfy's Phase II/IIIs could have benefited from functional unblinding (due to vomiting/nausea/GI AEs) and expectation bias (M1/M4 MOA was quite novel at the time), thereby widening its pbo-adj efficacy.

(b) Cleaner safety/tolerability means fewer/ less severe rates of vomiting and GI/urinary AE side effects, which technically could be dampening Cobenfy's uptake (US sales were $50M in Q4:25, up from $43M Q/Q). A real-world Cobenfy poster shows just 18% pts reaching the target dose (125/30mg), likely due to AEs (64% nausea, 37% vomiting) - with another 26% on the less efficacious starting dose (50/20mg). In another Cobenfy poster, 70-75%+ of OLE patients dropped out by Week 52. **(c) Better convenience** means QD (vs BID), no fasting needed (MPLT pts can still take with food), and a short 1-day titration (vs 1-2 weeks for Cobenfy).

FY (Dec)	2024A	2025E	2026E	2027E
EPS	(105.37)	(9.95)	(3.82)	(4.99)
Rev. (MM)	0.0	0.0	0.0	0.0
FY P/E	NM	NM	NM	NM

ESTIMATE CHANGE

RATING	BUY
PRICE	$19.07^
PRICE TARGET | % TO PT	$32.00 | +68%
52W HIGH-LOW	$21.55 - $12.24
FLOAT (%) | ADV MM (USD)	24.3% | 3.01
MARKET CAP	$964.9M
TICKER	MPLT

^Prior trading day's closing price unless otherwise noted.

FY (Dec)	CHANGE TO JEFe		JEF vs CONS	
	2025	2026	2025	2026
REV	NA	NA	NA	NA
EPS	NA	<1%	NA	NA

2025 ($)	Q1A	Q2A	Q3A	Q4	FY
EPS	(29.01)	(39.53)	(37.18)	(0.82)	(9.95)
PREV					

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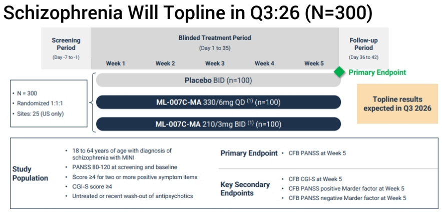
Strong Phase II study execution. (1) Mgmt extensively vetted the 25 US study sites by assessing which ones have contributed to (+) SCZ outcomes, **(2)** The distribution of patients per site is spread evenly, **(3)** Enrolled patients are triple-eligibility checked by the PI investigator, a central reviewer and MPLT's own team, analyzing medical records and removing professional patients. **(4)** For SAEs, no news is good news. Dropout rates are tracking within mgmt's expectations.

Exhibit 1 - MPLT Expected Newsflow

Timing	Expected Newsflow	Drug	Indication
Q3:26	Phase II data (N=300) in schizophrenia (**Study 211; ZEPHYR**)	ML-007C-MA (M1/M4 Agonist)	Schizophrenia
Q3:26	Phase II data (N=160) in autism spectrum disorder (**IRIS**)	ML-004 (5-HT1B/1D Agonist)	Autism Spectrum Disorder (ASD)
H2:26	Complete IND-enabling studies for ML-021 in Parkinson's	ML-021 (M4 Antagonist)	Parkinson's Disease
2026	Initiate IND-enabling studies for hyperactivity/agitation	ML-009 (GPR52 PAM)	Hyperactivity/Impulsivity
H2:27	Phase II data (N=300) in ADP Alzheimer's psychosis (**Study 221; VISTA**)	ML-007C-MA	Alzheimer's Disease Psychosis (ADP)

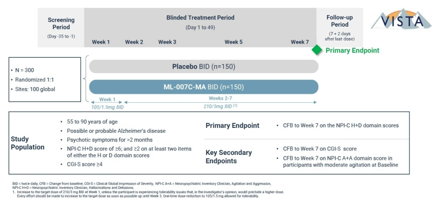
Source: Company reports, Jefferies estimates

Exhibit 2 - Phase II ZEPHYR Study of ML-007C-MA in Schizophrenia Will Topline in Q3:26 (N=300)



Source: Company reports

Exhibit 3 - MPLT's Phase II Study Design for ML-007C-MA in Alzheimer's Disease Psychosis is Set to Readout in H2:27



Source: Company reports

Exhibit 4 - Comparing MPL-007 Phase I Healthy Adult Data vs KRTX's Phase I Healthy Adult with KarXT (Cobenfy) - and KRTX's Data Evolution Into Pivotal Phase II/III SCZ Studies

Contact us for full safety tables

Drug / Phase / Safety Population	ML-007C-MA Phase 1 Healthy Volunteers		KarXT Phase 1 Healthy Volunteers		KarXT Phase 2 Schizophrenia Patients	KarXT Phase 3 Schizophrenia Patients	KarXT Phase 3 Schizophrenia Patients
Dose	210/3 mg BID (N=6)	330/6 mg QD (N=6)	100/20 mg BID (N=18)	125/40mg BID (N=18)	EMERGENT-1 (N=89)	EMERGENT-2 (N=126)	EMERGENT-3 (N=125)
Subjects with any TEAE	3 (50%)	4 (67%)	12 (67%)	16 (89%)	52%	74%	70%
Mild	2 (33%)	2 (33%)	9 (50%)	10 (56%)	35%	38%	36%
Moderate	1 (17%)	2 (33%)	3 (17%)	6 (33%)	16%	29%	26%
Severe	0 (0%)	0 (0%)	0 (0%)	0 (0%)	1%	7%	8%
Procholinergic TEAEs							
Nausea	3 (50%)	3 (50%)	4 (22%)	4 (22%)	17%	19%	19%
Vomiting	0%	0%	5 (28%)	1 (6%)	9%	14%	16%
Hyperhidrosis	1 (17%)	2 (33%)	1 (6%)	2 (11%)	2%	-	-
Salivary Hypersecrection	0%	-	1 (6%)	0%	-	-	-
Anticholinergic TEAEs							
Dry mouth	0%	-	2 (11%)	5 (28%)	9%	5%	0%
Dysuria	0%	-	0 (0%)	4 (22%)	-	-	-
Constipation	0%	-	1 (6%)	1 (6%)	17%	21%	13%
Urinary Retention	0%	-	0 (0%)	2 (11%)	-	-	-
Eye disorders	0%	-	0 (0%)	5 (28%)	-	5%	-
Other TEAEs							
Dizziness	3 (50%)	2 (33%)	4 (22%)	7 (39%)	3%	9%	0%
Postural dizzines	0%	-	2 (11%)	2 (11%)	0%	0%	0%
Headache	0%	-	3 (17%)	4 (22%)	7%	13%	11%

Source: Jefferies, Company reports

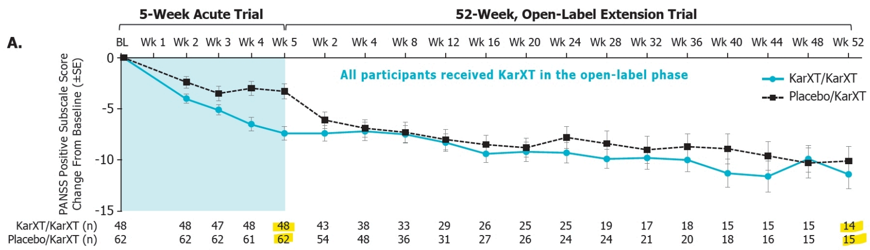
Exhibit 5 - Cross-Trial Efficacy Comparison of Muscarinics in Schizophrenia

Contact us for full tables across every SCZ drug and time point

Drug/Study Information							4-6 Week PANSS*			
Company	Name	Molecule	MOA	Status	Study	Study Phase	BL	Drug	Pbo	Pbo-adj
KRTX, BMY	Cobenfy	xanomeline + trospium Cl	M1-M4 + anti-cholinergic	Approved 2024	*KAR-004*	II	-	-17.4	-5.9	**-11.5**
					KAR-007	III	98.2	-21.2	-11.6	**-9.6**
					KAR-009	III	96.5	-20.6	-12.2	**-8.4**
NBIX	NBI-1117568	direclidine	Selective M4 orthosteric agonist	Phase III	*SCZ2028*	II	97.0	-18.2	-10.8	**-7.4**
CERE, ABBV	CVL-231	emraclidine	Selective M4 receptor PAM	Phase II	*SCH-001*	Ib	93.0	-19.5	-6.8	**-12.7**
					EMPOWER-1	II	98.3	-16.5	-13.5	**-3.0**
					EMPOWER-2	II	97.2	-14.2	-16.1	**1.9**
						AVERAGE:	**96.7**	**-18.2**	**-11.0**	**-7.2**
						MEDIAN:	**97.1**	**-18.2**	**-11.6**	**-8.4**

Source: Jefferies, Company reports, FDA labels

Exhibit 6 - Cobenfy OLE Data Shows 70-75% Discontinuation Rates Over 52 Weeks



Source: Company reports

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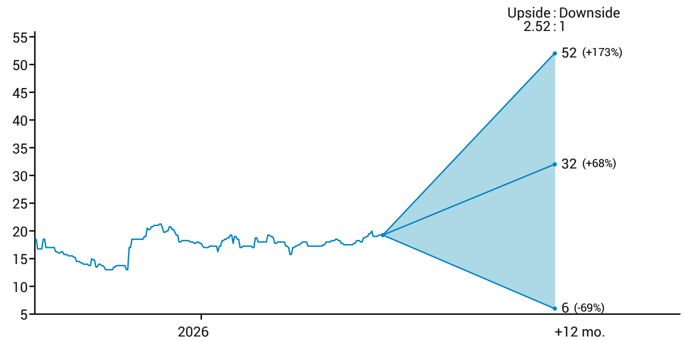
The Long View: Maplight Therapeutics

Investment Thesis

Maplight Therapeutics (MPLT) is a clinical-stage biotech company focused on improving outcomes for patients with central nervous system (CNS) disorders, including schizophrenia. Their lead compound, ML-007C-MA (M1/M4) could offer a differentiated approach to treatment in both schizophrenia and Alzheimer's disease psychosis (ADP) by potentially improving dosing convenience, as well as safety/efficacy. Schizophrenia and ADP are both $1B + opportunities in theory, making the risk/reward favorable at this valuation. MPLT also has a pipeline of other CNS/Neuro drugs for $1B+ markets that are in Phase II trials (e.g., ML-004 in ASD) or could enter the clinic as early as 2026-27 to offer additional valuation support/upside.

Risk/Reward - 12 Month View



Base Case, $32, +68%

- Our base case is a SOTP analysis of ML-007C-MA in schizophrenia ($1-2B peak sales and 30-40% PoS) and ADP ($1-2B peak sales and 10-20% PoS), as well as $500M-1B+ peak sales for ML-004 in ASD and 10-20% PoS.
- No credit is given for ML-021 in Parkinson's or ML-009 in hyperactivity/impulsivity
- For each program, we build the revenue and cash flow streams out to patent term expirations before probability-adjusting our DCF.
- Some credit given for cash

Upside Scenario, $52, +173%

- Our upside case includes a higher PoS for lead program ML-007C-MA in schizophrenia (55-65% PoS) and ADP (25-35% PoS), as well ASD (15-25%) and earlier-stage pipeline programs (0-10%)
- No credit given for cash

Downside Scenario, $6, -69%

- Our downside case considers a lower PoS for lead program ML-007C-MA in schizophrenia (0-10%) and no credit for ADP. No credit for earlier-stage pipeline programs.
- Some credit given for cash

Sustainability Matters

Top Material Issue(s): 1) Product Quality and Safety: Lead drug ML-007C-MA could have a differentiated therapeutic profile than other epilepsy drugs, including no titration, a differing AE profile, and no food effect. MPLT should continue to demonstrate ML-007C-MA is safer and more convenient than its competitors, as this is a major point of differentiation. **2) Employee Engagement, Diversity and Inclusion:** Smaller biotech companies must ensure that they have sufficient talent to develop drugs (whereas large pharma/biotech may provide more stability). This is especially critical as companies look to supplement their commercial portfolio with newer drugs and or BD opportunities.

Company Target(s): None

Qs to Mgmt: 1) What gives you confidence in the safety profile of your schizophrenia and ADP drug vs competitors, including Cobenfy? **2)** What steps have you taken to prepare for 3+ Phase II studies in CNS/Neuro indications in the form of relavent employee and/or Board hires?

Catalysts

- **Q3:26:** Phase II **ZEPHYR** data for ML-007C-MA in schizophrenia
- **Q3:26:** Phase II **IRIS** data for ML-004 in autism spectrum disorder (ASD)
- **2026:** IND updates for ML-021 in Parkinson's disease and ML-009 in hyperactivity/impulsivity
- **H2:27:** Phase II **VISTA** data for ML-007C-MA in Alzheimer's disease psychosis (ADP)

ML-007C-MA for Schizophrenia

Schizophrenia Phase II (ZEPHYR)

In Q3:26, ML-007C-MA could show differentiated Phase II SCZ data in Q3:26. MPLT mgmt characterized ZEPHYR as intentionally conventional in structure, engineered for credibility. **(1)** ZEPHYR (N=300) randomizes pts 1:1:1 to 210/3mg BID, 300/6mg QD or pbo, testing a standard Week 5 PANSS as the primary endpoint. **(2)** The in-patient study has N=25 sites, all based in the US to mitigate elevated pbo responses, which can be common in geographies with less infrastructure and experience in running schizophrenia trials (e.g., CERE). **(3)** MPLT's base case is to show **(a) competitive efficacy** to BMY's xanomeline (M1/M4)+trospium (approved as Cobenfy), which saw absolute PANSS benefits of -20-21 vs -11-12 pbo in EMERGENT-2/3 (or -8-10 point pbo-adj), plus **(b) better safety/tolerability** for both the BID and QD dose regimens, and **(c) better ease-of-use** (e.g., no fasting needed, shorter titration, QD dosing). **(4)** We think all 3 attributes are interconnected. **(5)** On efficacy, MPLT's stronger M1/M4 agonism means a greater chance to benefit all SCZ domains (including negative symptoms).

- **Safety**: MPLT notes that its Phase I did not see any anticholinergic AEs, though it would not be surprised to see some anticholinergic AEs in the Phase II given its longer study. Mgmt frames the situation as procholinergic being a tolerability issue, and anticholinergic as a safety issue (e.g., constipation/urinary retention can result in ER visits). As such, the study is slightly procholinergic, as most patients tolerate mild nausea/vomiting.
- **Dose convenience**: Both active BID and QD doses are modelled to achieve CSF exposures capable of driving benefit and, based on MPLT's preclinical PK/PD correlation models, should have 10-30-fold greater efficacy by dose than competing muscarinics. Both active dose arms are independently powered (>90%) to detect an effect size that is a discount to what Cobenfy showed in its Phase 2/3 study, with alpha "spent at the highest level" across the two active vs pbo comparisons, then 90% powered to show separation between the active doses (secondary analysis to the primary alpha).
- **Efficacy:** MPLT points out Cobenfy showed a significant effect size comparable to most approved antipsychotics on the market today (-8-10 point pbo-adj PANSS), which it views as the efficacy target (base case), with emphasis on real-world adherence/persistence of patients staying on drug as the key differentiator. Even so, MPLT's higher exposures could potentially drive better efficacy (upside scenario).

ZEPHYR remains on track per safety guardrails in place. MPLT offered a soft, bounded reassurance on blinded safety, stating that if there were a significant departure from Phase I expectations, it would have shared it, while acknowledging that no blinded safety detail has been disclosed publicly. Discontinuation rates are also falling within its expectations. Also, allowing dropdown doses only reduces the agonist dose, while the anticholinergic dose remains the same, further skewing the profile towards a procholinergic profile.

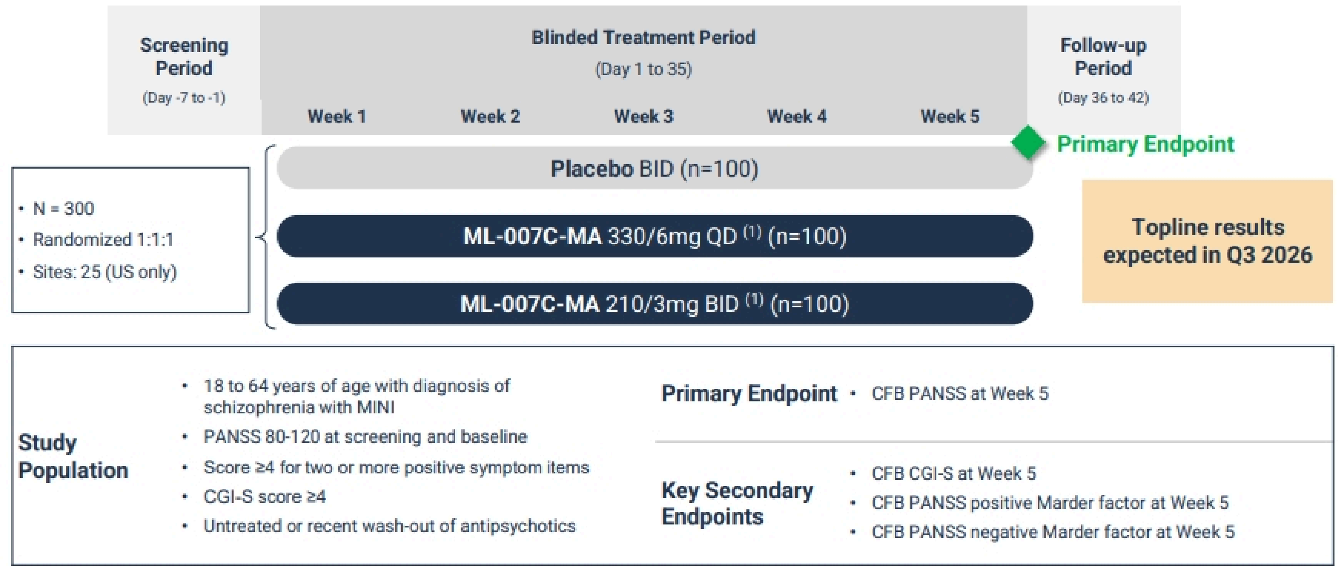
The schizophrenia patient population in the study reflects that of typical schizophrenia trials, with slight enrichment of severe patients to pot'ly increase the probability of detecting improvement. Patients are triple-eligibility checked by **(1)** the study investigator, **(2)** a central reviewer, and **(3)** the MPLT team, which also reviews previous screening, prior enrollment attempts at other sites, and prior participation in other muscarinic or antipsychotic clinical trials (or if they were rejected). Subjects who participated in prior muscarinic studies must have responded.

Study site selection for ZEPHYR underwent an independent assessment in which MPLT reviewed all the reviewer data (e.g., prior trial participation, success concordance, etc) to select its N=25 US-based sites for the study, ~75-80% of which overlap with Coebenfy's studies. Even with the limited number of sites, enrollment to N=300 is relatively quick (~12 months), even though each site was capped to about N=2 patients per month to ensure no slippage in study integrity. Some sites were stopped when they hit thresholds, and a few higher-performing sites were allowed to exceed initial allocations modestly.

Other tidbits

- Patients are instructed to take ML-007C-MA with food (could be any type), which increases exposure. This is supported by safety/tolerability data for ML-007C-MA, which suggests no meaningful difference between a fed and fasted state.
- About 30%+ of schizophrenia patients on D2s are on multiple D2s, though there are no conclusive data on whether the effects would be additive.
- Adjunctive therapy in schizophrenia is quite common. That said, drugs approved in multiple indications (e.g., Caplyta for schizophrenia and bipolar depression) could help cater to patient subsets with additional symptoms more appropriately.

Exhibit 7 - Phase II ZEPHYR Study of ML-007C-MA in Schizophrenia Will Topline in Q3:26 (N=300)



Source: Company reports

Open-label long-term extension study starts imminently for ML-007C-MA

A long-term open label extension (OLE) study was recently posted on ct.gov and is expected to start imminently (N=500; NCT07459647). The OLE will help accrue safety exposure data (gaiting factor) to support a future NDA filing. Mgmt suggested rollover from acute inpatient schizophrenia trials is typically low because incentives differ, especially "professional patients," who are less likely to continue into outpatient long-term follow-up. As such, MPLT will skew initial enrollment toward de novo sites to build exposure data more quickly. For reference, Cobenfy's OLE enrolled N=700 to get to 100+ exposures out to a year by its mid-review cycle, exhibiting the challenges with aligning patient incentives vs inpatient studies.

Manufacturing, nonclinical, and timing readiness

MPLT invested in its CMC at risk for about 1.5 years, which now positions itself to move quickly into Phase III and then an NDA-enabling package if Phase II data are (+). Meanwhile, nonclinical tox work is "pretty much completed" other than a 2-year carcinogenicity study, though potential pathways to acceleration (e.g., waivers) could be an option, as MPLT plans to ask for all acceleration options.

MPLT has completed longer animal toxicity work for up to 13 weeks at high doses without any signals. MPLT expects cholinergic AEs and some weight loss in animals. They framed the FDA's comfort in allowing longer-term human exposure work as indirect support for the view that the package is not raising major concerns.

ML-007C-MA for ADP

The Phase II VISTA trial of ML-007C-MA for Alzheimer's Psychosis (ADP)

The VISTA Phase II trial is a double-blind, placebo-controlled study evaluating ML-007C-MA for the treatment of hallucinations and delusions associated with Alzheimer's Disease Psychosis (ADP), with topline data expected in H2:27. (1) The trial aims to enroll approximately N=300 participants globally, who will be randomized in a 1:1 ratio to receive either ML-007C-MA at a dose of 210/3mg twice daily or placebo. **(2)** Patients begin treatment with a titration dose of ML-007C-MA 105/1.5 mg twice daily (or placebo) for one week before transitioning to the maintenance dose (e.g., 210/3mg). **(3)** To address tolerability concerns, participants will be permitted a one-time dose reduction; those requiring this adjustment will remain on the reduced dose for the remainder of the study. **(4)** The primary endpoint is changed from baseline to Week 7 in the Neuropsychiatric Inventory-Clinician Hallucinations and Delusions (NPI-C H+D) score, with secondary measures including Changes in Clinical Global Impression (e.g., CGI) scores and NPI-C agitation and aggression scores. **(5)** There are technically no approved drugs for ADP, making determining meaningful efficacy difficult to handicap.

ADP Phase II (VISTA), and links to competitor issues vs its own controls

MPLT remains cautious about reading too much into BMY's Cobenfy ADP program operational issues and what the re-enrollment may mean for efficacy. Briefly, BMY will now upsize Cobenfy's Phase III ADP study (ADEPT-2), prolonging the readout to YE:26 (vs Q4:25 previously). **(1)** The decision follows a review of clinical trial sites that revealed irregularities in clinical trial execution at a small number of sites. As such, BMY decided to exclude certain patient data and upsize the study after consulting with the DMC and the FDA as well as reviewing patient data (on a blinded basis). **(2)** We wonder if the study could be showing a modest (+) efficacy signal (creating room for MPLT to differentiate on efficacy/safety/convenience), arguably better than expectations.

MPLT mgmt emphasized as evidence that site conduct risk is real, and FDA's willingness to exclude data from select sites may imply something "egregious," corroborating our view that the risk of data integrity seems driven more by study execution than the underlying MOA. Said differently, BMY's situation could be idiosyncratic (i.e., MPLT is unaffected).

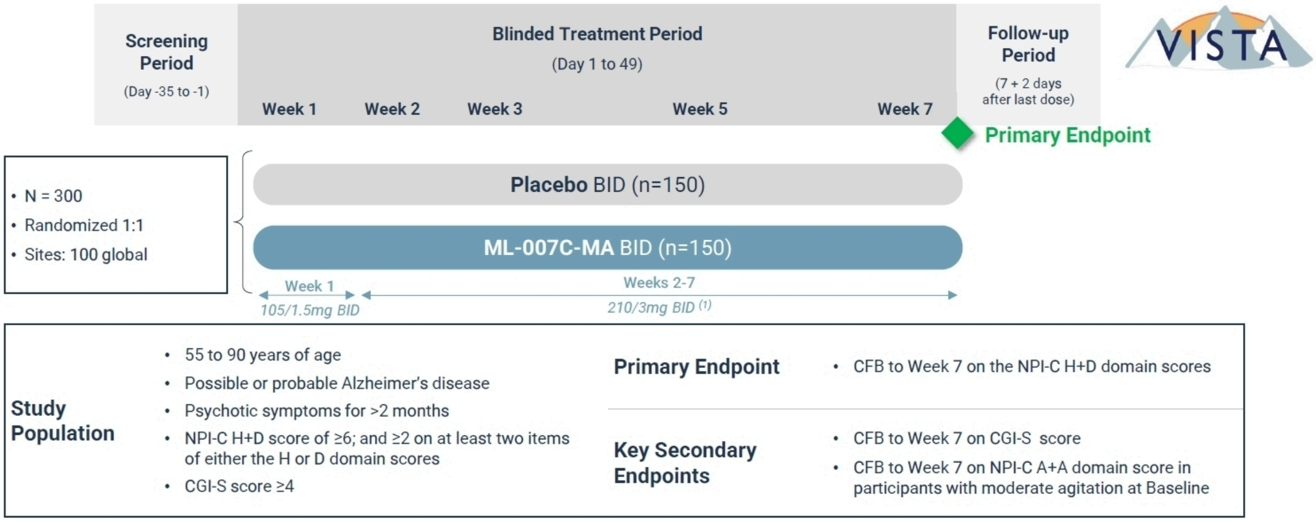
Mgmt used that competitor situation to highlight MPLT's own monitoring and quality framework, emphasizing that it has brought in 100% of its site monitoring in house, use of only US study sites, and having each of its MPLT-employed clinical research associates (CRA) be responsible for a single study and only handful (~4-5) sites, and provide a high level of involvement with study conduct. Add'ly, subsequent to ADEPT-2's disclosures, MPLT implemented more quality measures to assure study integrity.

- For context, on study designs, BMY's ADEPT-2 (originally N=400) is randomizing patients 1:1 to drug vs pbo with a 5-week titration phase (20/2mg, 30/3mg, 40/4mg, 50/5mg, 66.7/6.7mg), and the primary endpoint is NPI-C (H+D) at Week 14. MPLT's VISTA study (N=300) utilizes a simple 1-week titration (105/1.5mg BID to 210/3mg BID) with the same primary but at Week 7.

We remain constructive on ADP, considering the high unmet need, as well as early clinical trial results for psychosis symptoms associated with Alzheimer's. In theory, an approved safety profile (for both

Cobenfy and ML-007C-MA) compared to xanomeline alone could improve trial execution, given the earlier 6-month trial's high 52% discontinuation rate.

Exhibit 8 - MPLT's Phase II Study Design for ML-007C-MA in Alzheimer's Disease Psychosis is Set to Readout in H2:27



BID = twice daily, CFB = Change from baseline, CGI-S = Clinical Global Impression of Severity, NPI-C A+A = Neuropsychiatric Inventory Clinician, Agitation and Aggression, NPI-C H+D = Neuropsychiatric Inventory Clinician, Hallucinations and Delusions.
1. Increase to the target dose of 210/3 mg BID at Week 1, unless the participant is experiencing tolerability issues that, in the investigator's opinion, would preclude a higher dose. Every effort should be made to increase to the target dose as soon as possible up until Week 3. One-time dose reduction to 105/1.5 mg allowed for tolerability.

Source: Company reports

Exhibit 9 - BMY Phase III Program in Alzheimer's Disease Psychosis

Indication	Psychosis in Alzheimer's Disease (ADP)	
Phase/Study	Phase III - ADEPT-1	Phase III - ADEPT-2
# of Patients	N = 380	N = 400
Design	• Cobenfy 20 mg/2 mg TID, 30 mg/3 mg TID, 40 mg/4 mg TID, 50 mg/5 mg TID, 66.7/6.67 mg TID* • Placebo	• Cobenfy 20 mg/2 mg TID, 30 mg/3 mg TID, 40 mg/4 mg TID, 50 mg/5 mg TID, 66.7/6.67 mg TID* • Placebo
Endpoints	• Primary: Time from randomization to relapse during the 26-week double blind randomized withdrawal period • Key secondary: Time from randomization to discontinuation for any reason during the 26-week Double-Blind Randomized Withdrawal treatment Period	• Primary: Change from baseline in Neuropsychiatric Inventory-Clinician: Hallucinations and Delusions (NPI-C: H+D) score to end of Week 14 • Key secondary: Change from baseline in Clinical Global Impressions-Severity (CGI-S) scale up to week 14.
Status	• Recruiting • Projected data readout 2026	• Recruiting • Projected data readout 2026
CT Identifier	NCT05511363	NCT06126224

*Based-on tolerability

Source: Company reports

ML-007C-MA in Additional Indications

Cognition Angle and Future Indication Expansion

MPLT suggests cognition as an underappreciated part of the MPLT story, arguing that stronger M1 engagement could translate into cognitive benefits. Even if the acute schizophrenia trial (ZEPHYR) is not designed to support a cognition label directly, MPLT is prospectively defining cognitive domains in the study to support publication-quality learning as opposed to post hoc analyses. In fact, MPLT is selecting a third indication for ML-007C-MA that is cognition-oriented, which it plans to disclose at a later date in the future.

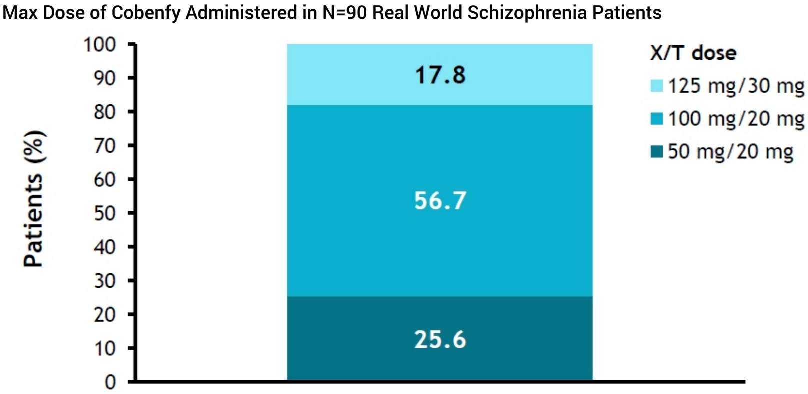
Mechanistic rationale is further supported by LLY's Phase II study in 1997 for Alzheimer's that showed xanomeine high dose exhibiting a statistically significant treatment effect on ADAS-Cog ($p<0.05$), but was burdened by tolerability issues (52% patientd discontineud traetment). Also, TAK recently published Phase II results of TAK-071 (M1 receptor-positive allosteric modulator) showing improvements on a cognitive composite score as a secondary endpoint in its Phase II for Parkinson's disease with cognitive impairment. MPLT envisions its study in cognition to be open to background use of acetylcholinesterase inhibitors, as the mechanisms could be complementary.

How Mgmt Discussed Cobenfy and Why it Matters to Differentiation

MPLT's sharpest practical point was that Cobenfy's real-world experience may be less consistent with what the controlled inpatient trials data suggests, and argues that this gap is driven by tolerability burden, dosing workflow complexity, and PK variability that are hard to manage outside the inpatient setting. MPLT mgmt pointed to a real-world dataset presented at Psych Congress 2025 on N=90 patients treated with Cobenfy, which suggested near 100% adjunctive use, <20% of patients reaching the higher 125/30mg dose, and >50% antiemetic use, yet still 64% of patients reported nausea and 37% reported vomiting. MPLT's own market research and claims-based observations suggest <20% of patients remain on Cobenfy by 12 months, which it frames as a key limiter of the sales trajectory because persistence matters as much as new starts.

MPLT explains some of Cobenfy's tolerability challenges may be attributed to PK mismatch (xanomeline 0 to 30% higher Cmax fed vs fasted, trospium 70 to 90% lower), which can lead to inconsistent peripheral antagonist "coverage" relative to agonist exposure, creating a moving target where excess agonist yields pro-cholinergic AEs while excess antagonist yields anti-cholinergic AEs, and they presented that as a fundamental reason to believe real-world tolerability will be harder than trial optics.

Exhibit 10 - Max Dose of Cobenfy Administered in N=90 Real World Schizophrenia Patients



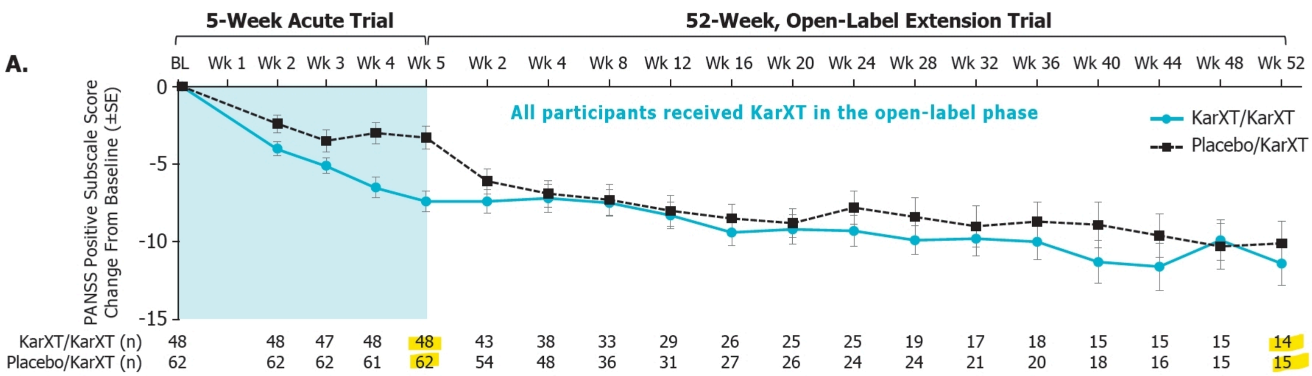
X/T, xanomeline/trospium.
Source: Company reports

Parameter(1)(2)	Xanomeline M_1/M_4 Agonist	Trospium Peripheral Antagonist
Oral Bioavailability	~1%	~15%
Plasma Protein Binding	95%	80%
T_{max}	~2.0 hours	~0.7 hours
Coefficient of Variation	~94-107%	~88-94%
Fed vs. Fasted (C_{max})	0-30% higher	70-90% lower

Significant Intra- and Inter-patient Variability Results in PK Mismatch

- × Excess agonist → pro-cholinergic AEs (vomiting, nausea, diarrhea, etc.)
- × Excess antagonist → anti-cholinergic AEs (urinary retention, constipation, etc.)

Exhibit 11 - Cobenfy OLE Data Shows 70-75% Discontinuation Rates Over 52 Weeks



Source: Company reports

Exhibit 12 - Comparing MPL-007 Phase I Healthy Adult Data vs KRTX's Phase I Healthy Adult with KarXT (Cobenfy) - and KRTX's Data Evolution Into Pivotal Phase II/III SCZ Studies

Contact us for full safety tables

Drug Phase Safety Population	ML-007C-MA Phase 1 Healthy Volunteers		KarXT Phase 1 Healthy Volunteers		KarXT Phase 2 Schizophrenia Patients	KarXT Phase 3 Schizophrenia Patients	KarXT Phase 3 Schizophrenia Patients
Dose	210/3 mg BID (N=6)	330/6 mg QD (N=6)	100/20 mg BID (N=18)	125/40mg BID (N=18)	EMERGENT-1 (N=89)	EMERGENT-2 (N=126)	EMERGENT-3 (N=125)
Subjects with any TEAE	3 (50%)	4 (67%)	12 (67%)	16 (89%)	52%	74%	70%
Mild	2 (33%)	2 (33%)	9 (50%)	10 (56%)	35%	38%	36%
Moderate	1 (17%)	2 (33%)	3 (17%)	6 (33%)	16%	29%	26%
Severe	0 (0%)	0 (0%)	0 (0%)	0 (0%)	1%	7%	8%
Procholinergic TEAEs							
Nausea	3 (50%)	3 (50%)	4 (22%)	4 (22%)	17%	19%	19%
Vomiting	0%	0%	5 (28%)	1 (6%)	9%	14%	16%
Hyperhidrosis	1 (17%)	2 (33%)	1 (6%)	2 (11%)	2%	-	-
Salivary Hypersecretion	0%	-	1 (6%)	0%	-	-	-
Anticholinergic TEAEs							
Dry mouth	0%	-	2 (11%)	5 (28%)	9%	5%	0%
Dysuria	0%	-	0 (0%)	4 (22%)	-	-	-
Constipation	0%	-	1 (6%)	1 (6%)	17%	21%	13%
Urinary Retention	0%	-	0 (0%)	2 (11%)	-	-	-
Eye disorders	0%	-	0 (0%)	5 (28%)	-	5%	-
Other TEAEs							
Dizziness	3 (50%)	2 (33%)	4 (22%)	7 (39%)	3%	9%	0%
Postural dizzines	0%	-	2 (11%)	2 (11%)	0%	0%	0%
Headache	0%	-	3 (17%)	4 (22%)	7%	13%	11%

Source: Jefferies, Company reports

Exhibit 13 - Cross-Trial Efficacy Comparison of Muscarinics in Schizophrenia
Contact us for full tables across every SCZ drug and time point

Drug/Study Information							4-6 Week PANSS*			
Company	**Name**	**Molecule**	**MOA**	**Status**	**Study**	**Study Phase**	**BL**	**Drug**	**Pbo**	**Pbo-adj**
KRTX, BMY	Cobenfy	xanomeline + trospium Cl	M1-M4 + anti-cholinergic	Approved 2024	*KAR-004*	II	-	-17.4	-5.9	**-11.5**
					KAR-007	III	98.2	-21.2	-11.6	**-9.6**
					KAR-009	III	96.5	-20.6	-12.2	**-8.4**
NBIX	NBI-1117568	direclidine	Selective M4 orthosteric agonist	Phase III	*SCZ2028*	II	97.0	-18.2	-10.8	**-7.4**
CERE, ABBV	CVL-231	emraclidine	Selective M4 receptor PAM	Phase II	*SCH-001*	Ib	93.0	-19.5	-6.8	**-12.7**
					EMPOWER-1	II	98.3	-16.5	-13.5	**-3.0**
					EMPOWER-2	II	97.2	-14.2	-16.1	**1.9**
						AVERAGE:	**96.7**	**-18.2**	**-11.0**	**-7.2**
						MEDIAN:	**97.1**	**-18.2**	**-11.6**	**-8.4**

Source: Jefferies, Company reports, FDA labels

MPLT comments on other muscarinic programs

- NBIX's muscarinic assets are viewed as coming from a common GPCR engineering portfolio rather than being fundamentally distinct programs optimized independently by indication. True orthosteric muscarinic subtype selectivity is seen as extremely difficult due to conserved binding sites across M1–M5, so apparent selectivity is interpreted as a consequence of weak partial agonism rather than superior design. In this framing, claims that a peripheral antagonist is unnecessary reflect insufficient receptor engagement, not a solved tolerability problem.
- When benchmarked in comparable preclinical models, these compounds were described as the least effective at concentrations plausibly achievable in humans, reinforcing the view that efficacy is being traded off to avoid peripheral liability. Small Phase II datasets, often with N=40-50 patients per arm across many sites, were characterized as difficult to interpret and poor predictors of Phase III outcomes, even if statistical significance is achieved.
- An M1-preferring approach was acknowledged as potentially logical in cognition-focused settings such as ADP or elderly populations.

ML-004 for ASD

ML-004 (oral 5-HT1B/1D) has 12-week Phase II ASD data (IRIS) in Q3:36 as well (N=160+). (a) The exploratory trial will randomize N=60 adults and N=100 adolescents to flexible 48-72mg doses of '004 or pbo, assessing the ABI-SC social communication scale (licensed from JNJ). **(b)** Key secondaries include the ABC-I irritability scale, which was leveraged by approved antipsychotics **(c)** Patients with social communication deficits are a larger market (albeit higher risk), whereas patients with irritability are a smaller market (still meaningful though), but the regulatory pathway is established. **(d)** We think mgmt has optionality to run registrational trials in either (or both) ASD symptoms.

Exhibit 14 - Phase II IRIS Study of ML-004 in Autism Spectrum Disorder Will Topline in Q3:26 (N=160)

Source: Company reports

MPLT Framing

MPLT mgmt emphasized lead asset ML-007C-MA is a dual M1/M4 muscarinic agonist co-formulated with a peripheral antagonist that it believes can preserve muscarinic efficacy while materially improving real-world tolerability, dosing practicality, and PK consistency vs first-wave approaches. MPLT mgmt anchored that positioning in a broader company narrative built around circuit-driven target discovery, but it was explicit that investor focus is concentrated in the clinical muscarinic program, with ML-004 as the next most visible near-term catalyst and additional preclinical disclosures expected over the coming months.

Company overview and pipeline context

MPLT mgmt emphasized that the company was founded in **2019** to pursue circuit-specific therapies in CNS disorders, and it described a discovery engine that has generated a "broad pipeline" spanning clinical and preclinical programs. MPLT mgmt repeatedly referenced its clinical muscarinic program **ML-007C-MA (**M1/M4 agonist co formulated with a peripherally acting anticholinergic **fesoterodine**) as its core value driver. The rest of the pipeline adds both diversification and optionality: **ML-004** (5-HT1B/1D agonist) in Phase II for autism spectrum disorder, **ML-021** (M4 antagonist) in preclinical development for Parkinson's disease, and **ML-009** (GPR52 PAM) as a nominated preclinical candidate for hyperactivity and impulsivity.

Core scientific thesis on muscarinics

MPLT mgmt's most repeated scientific assertion was that robust schizophrenia efficacy at clinically usable doses requires engaging both M1 and M4 centrally. Mgmt argues that single receptor strategies may theoretically work, but likely only at higher exposures that become clinically problematic. MPLT mgmt also insists that any centrally active muscarinic approach will inevitably drive peripheral muscarinic activity, which it views makes peripheral AEs dose-limiting and creates the need for a peripheral antagonist to "offset" cholinergic toxicity.

MPLT framed its muscarinic thesis around three core beliefs:

- **(1)** Robust central M1 <u>and</u> M4 receptor engagement likely requires higher exposures that are easy to sustain clinically. Achieving therapeutic efficacy by targeting either receptor (e.g., M4-based approaches in the case of ABBV/CERE emraclidine and NBIX's direclidine) is possible (albeit results are mixed), but requires dosing at very high levels, which is challenging clinically (due to potential side effects).
- **(2)** Any centrally active muscarinic approach will also drive peripheral muscarinic activation that becomes dose-limiting without proper mitigation. For emraclidine, the Phase I 30mg dose was in part carried forward in the Phase II trials (which failed). ABBV showed moderate cardiovascular AEs. Net-net, a peripheral antagonist is necessary to offset peripheral M1/M4 muscarinic activity.
- **(3) KarXT** (Cobenfy, xanomeline + trospium combination) has fundamental challenges with the drug itself. Xanomeline (despite having clear biological activity) has <1% bioavailability and is a high protein and lipid-bound molecule (e.g., a low free-fraction), contributing to inter and intra-patient variability. Phase I healthy volunteer data CVs are "north of 100%"

ML-007C-MA could be a "best of both worlds" attempt to combine M1 and M4 activity at lower clinically required exposure than either alone, while using a peripheral antagonist approach to blunt peripheral muscarinic adverse effects, similar in concept to Karuna's strategy. They also emphasized trial execution, including US-only sites, tight enrollment controls, and in-house monitoring to avoid conduct issues that may have affected some recent muscarinic programs.

Financial position and runway

MPLT has approximately $450M as of December 31, with Q4:25 EPS results expected to formalize the figure and runway into 2027, while also stressing that this runway assumes meaningful spending, including Phase III startup, CMC at risk, potentially another Phase II, and a third indication for ML-007C-MA.

Exhibit 15 - MPLT Expected Newsflow

Timing	Expected Newsflow	Drug	Indication
Q3:26	Phase II data (N=300) in schizophrenia (**Study 211; ZEPHYR**)	ML-007C-MA (M1/M4 Agonist)	Schizophrenia
Q3:26	Phase II data (N=160) in autism spectrum disorder (**IRIS**)	ML-004 (5-HT1B/1D Agonist)	Autism Spectrum Disorder (ASD)
H2:26	Complete IND-enabling studies for ML-021 in Parkinson's	ML-021 (M4 Antagonist)	Parkinson's Disease
2026	Initiate IND-enabling studies for hyperactivity/agitation	ML-009 (GPR52 PAM)	Hyperactivity/Impulsivity
H2:27	Phase II data (N=300) in ADP Alzheimer's psychosis (**Study 221; VISTA**)	ML-007C-MA	Alzheimer's Disease Psychosis (ADP)

Source: Company reports, Jefferies estimates

Exhibit 16 - MPLT Income Statement

Fiscal Year (Ending in December) ($ figures in MM, except per share)	FY23	1Q24 Mar-24	2Q24 Jun-24	3Q24 Sep-24	4Q24 Dec-24	FY24	1Q25 Mar-25	2Q25 Jun-25	3Q25E Sep-25	4Q25E Dec-25	FY25E	1Q26E Mar-26	2Q26E Jun-26	3Q26E Sep-26	4Q26E Dec-26	FY26E	FY27E	FY28E	FY29E	FY30E
GAAP Income Statement																				
Revenue																				
Net Product Revenue	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	$35.0
ML-007C-MA - Total	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	35.0
Schizophrenia	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	35.0
Alzheimer's Disease Psychosis	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-
ML-004 - Autism Spectrum Disorder	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-
ML-021 - Parkinson's Disease	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-
ML-009 - Hyperactivity/Impulsivity																				
Other Revenue	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-
Licensing revenue	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-
Other revenue	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-
Total Revenues	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	$35.0
Cost of Sales	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	1.8
% of Product Revenue	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%	5.0%
Operating Expenses	$57.3	$19.3	$20.0	$20.9	$22.8	$82.9	$23.5	$30.7	$31.5	$38.8	$124.5	$43.4	$48.0	$53.6	$60.2	$205.2	$302.0	$341.2	$385.8	$436.6
Research and Development (R&D)	49.7	15.3	15.6	16.8	20.7	68.5	19.8	26.8	27.1	34.0	107.7	38.0	42.0	47.0	53.0	180.0	271.9	305.2	342.7	385.0
Sales, General, and Administrative (SG&A)	7.6	3.9	4.3	4.1	2.1	14.4	3.8	3.8	4.4	4.8	16.8	5.4	6.0	6.6	7.2	25.2	30.1	36.0	43.1	51.5
Other Expenses	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-
Total Expenses	$57.3	$19.3	$20.0	$20.9	$22.8	$82.9	$23.5	$30.7	$31.5	$38.8	$124.5	$43.4	$48.0	$53.6	$60.2	$205.2	$302.0	$341.2	$385.8	$438.3
Operating Income (EBIT)	($57.3)	($19.3)	($20.0)	($20.9)	($22.8)	($82.9)	($23.5)	($30.7)	($31.5)	($38.8)	($124.5)	($43.4)	($48.0)	($53.6)	($60.2)	($205.2)	($302.0)	($341.2)	($385.8)	($403.3)
Total Other Income	1.6	0.4	1.6	1.9	1.5	5.4	1.2	0.8	2.1	1.8	5.9	4.5	5.8	5.4	6.0	21.7	24.7	26.1	17.4	24.2
Interest income	1.1	0.9	1.6	1.1	0.9	4.5	0.8	0.7	1.5	1.8	4.8	4.5	5.8	5.4	6.0	21.7	24.7	26.1	17.4	24.2
Interest expense	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-
Other income, net	0.5	(0.5)	(0.0)	0.8	0.6	0.9	0.4	0.1	0.5	-	1.1	-	-	-	-	-	-	-	-	-
Pre-Tax Income	(55.7)	(18.9)	(18.4)	(19.0)	(21.2)	(77.6)	(22.3)	(29.8)	(29.4)	(37.0)	(118.6)	(38.9)	(42.2)	(48.2)	(54.2)	(183.5)	(277.3)	(315.1)	(368.4)	(379.2)
Provision for taxes (benefit)	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-
Tax rate	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%	0.0%
Net Income	(55.7)	(18.9)	(18.4)	(19.0)	(21.2)	(77.6)	(22.3)	(29.8)	(29.4)	(37.0)	(118.6)	(38.9)	(42.2)	(48.2)	(54.2)	(183.5)	(277.3)	(315.1)	(368.4)	(379.2)
Other	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-
Net Income attributable to common stockholders	(55.7)	(18.9)	(18.4)	(19.0)	(21.2)	(77.6)	(22.3)	(29.8)	(29.4)	(37.0)	(118.6)	(38.9)	(42.2)	(48.2)	(54.2)	(183.5)	(277.3)	(315.1)	(368.4)	(379.2)
EPS (Basic)	($5.03)	($27.05)	($25.32)	($25.08)	($27.93)	($105.37)	($29.01)	($39.53)	($37.18)	($0.82)	($9.95)	($0.86)	($0.93)	($0.96)	($1.07)	($3.82)	($4.99)	($5.66)	($6.07)	($6.24)
EPS (Diluted, as reported)	($5.03)	($27.05)	($25.32)	($25.08)	($27.93)	($105.37)	($29.01)	($39.53)	($37.18)	($0.82)	($9.95)	($0.86)	($0.93)	($0.96)	($1.07)	($3.82)	($4.99)	($5.66)	($6.07)	($6.24)
Shares Outstanding - Basic	11.1	0.7	0.7	0.8	0.8	0.7	0.8	0.8	0.8	45.4	11.9	45.4	45.5	50.5	50.6	48.0	55.6	55.7	60.7	60.8
Shares Outstanding - Diluted (as reported)	11.1	0.7	0.7	0.8	0.8	0.7	0.8	0.8	0.8	45.4	11.9	45.4	45.5	50.5	50.6	48.0	55.6	55.7	60.7	60.8
Shares Outstanding - Fully Diluted + Preferred/Warrants	11.1	2.7	17.1	17.1	17.1	13.5	17.1	17.1	32.4	50.6	29.3	50.6	50.7	55.7	55.8	53.2	60.8	60.9	65.9	66.0
Shares Outstanding - Fully Diluted + Options/Warrants, etc	11.1	3.9	18.1	18.0	18.0	14.5	18.1	18.0	33.3	51.5	30.2	51.6	51.6	56.6	56.7	54.1	61.7	61.8	66.9	66.9

Source: Company reports, Jefferies estimates

Exhibit 17 - MPLT Balance Sheet

Fiscal Year (Ending in December) ($ figures in MM, except per share)	FY23	1Q24 Mar-24	2Q24 Jun-24	3Q24 Sep-24	4Q24 Dec-24	FY24	1Q25 Mar-25	2Q25 Jun-25	3Q25E Sep-25	4Q25E Dec-25	FY25E	1Q26E Mar-26	2Q26E Jun-26	3Q26E Sep-26	4Q26E Dec-26	FY26E	FY27E	FY28E	FY29E	FY30E
Current assets:																				
Cash and cash equivalents	79.8	179.8	38.7	19.7	38.3	38.3	50.2	33.5	64.1	324.4	324.4	286.7	246.0	393.4	341.0	341.0	436.1	130.9	258.7	(87.5)
Marketable securities	-	-	121.5	121.5	70.5	70.5	42.1	27.0	163.0	163.0	163.0	163.0	163.0	163.0	163.0	163.0	163.0	163.0	163.0	143.0
Prepaid expenses & other current assets	3.8	3.8	3.8	3.8	5.8	5.8	6.0	12.7	18.1	18.1	18.1	18.1	18.1	18.1	18.1	18.1	18.1	18.1	18.1	18.1
Restricted cash	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-
Accounts receivable	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-
Inventory	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-
Other current assets	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-
Total current assets	**83.6**	**183.6**	**164.1**	**145.1**	**114.6**	**114.6**	**98.3**	**73.2**	**245.3**	**505.6**	**505.6**	**467.9**	**427.2**	**574.6**	**522.1**	**522.1**	**617.3**	**312.0**	**439.9**	**73.7**
Marketable securities - long-term	-	0.0	0.3	0.3	11.4	11.4	2.5	-	-	-	-	-	-	-	-	-	-	-	-	-
Restricted cash - long-term	0.2	0.2	0.2	0.2	0.2	0.2	0.2	0.2	0.2	0.2	0.2	0.2	0.2	0.2	0.2	0.2	0.2	0.2	0.2	0.2
Other intangible assets (Right-of-use)	5.6	5.6	5.6	5.6	6.4	6.4	6.1	5.9	5.7	5.7	5.7	5.7	5.7	5.7	5.7	5.7	5.7	5.7	5.7	5.7
Property and equipment, net	1.0	1.0	1.0	1.0	1.2	1.2	1.0	1.0	0.9	0.9	0.9	0.9	0.9	0.9	0.9	0.9	0.9	0.9	0.9	0.9
Other long-term assets	1.0	1.0	1.0	1.0	3.2	3.2	3.4	3.8	5.1	5.1	5.1	5.1	5.1	5.1	5.1	5.1	5.1	5.1	5.1	5.1
Total assets	**91.4**	**191.4**	**172.2**	**153.2**	**136.9**	**136.9**	**111.6**	**84.1**	**257.2**	**517.5**	**517.5**	**479.8**	**439.1**	**586.5**	**534.0**	**534.0**	**629.2**	**323.9**	**451.8**	**85.6**
Current liabilities:																				
Accounts payable	3.3	3.3	3.3	3.3	1.9	1.9	0.9	2.1	2.4	2.4	2.4	2.4	2.4	2.4	2.4	2.4	2.4	2.4	2.4	2.4
Accrued expenses	6.8	6.8	6.8	6.8	10.0	10.0	8.1	9.6	14.0	14.0	14.0	14.0	14.0	14.0	14.0	14.0	14.0	14.0	14.0	14.0
Lease liability - current	0.5	0.5	0.5	0.5	0.8	0.8	0.8	0.8	0.9	0.9	0.9	0.9	0.9	0.9	0.9	0.9	0.9	0.9	0.9	0.9
Deferred grant earnings	4.2	4.2	4.2	4.2	3.2	3.2	3.2	2.4	1.2	1.2	1.2	1.2	1.2	1.2	1.2	1.2	1.2	1.2	1.2	1.2
Current portion of long-term debt	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-
Total current liabilities	**14.7**	**14.7**	**14.7**	**14.7**	**15.9**	**15.9**	**13.0**	**15.0**	**18.6**	**18.6**	**18.6**	**18.6**	**18.6**	**18.6**	**18.6**	**18.6**	**18.6**	**18.6**	**18.6**	**18.6**
Deferred revenue, net of current portion	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-
Long-term debt	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-
Preferred stock warrant liability	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-
Deferred tax liabilities	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-
Other long-term liabilities	5.4	5.4	5.4	5.4	5.8	5.8	5.6	5.4	5.1	5.1	5.1	5.1	5.1	5.1	5.1	5.1	5.1	5.1	5.1	5.1
Total liabilities	**20.1**	**20.1**	**20.1**	**20.1**	**21.7**	**21.7**	**18.6**	**20.4**	**23.7**	**23.7**	**23.7**	**23.7**	**23.7**	**23.7**	**23.7**	**23.7**	**23.7**	**23.7**	**23.7**	**23.7**
Stockholders' Equity:																				
Convertible preferred stock	188.9	188.9	188.9	188.9	308.8	308.8	308.8	308.8	507.7	507.7	507.7	507.7	507.7	507.7	507.7	507.7	507.7	507.7	507.7	507.7
Common stock	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-
Additional paid-in capital	4.2	4.2	4.2	4.2	5.6	5.6	5.8	6.4	6.8	6.8	6.8	6.8	6.8	6.8	6.8	6.8	6.8	6.8	6.8	6.8
Accumulated deficit and other	(121.8)	(21.8)	(41.0)	(60.0)	(199.2)	(199.2)	(221.6)	(251.5)	(281.0)	(20.7)	(20.7)	(58.4)	(99.1)	48.3	(4.2)	(4.2)	91.0	(214.2)	(86.4)	(452.6)
Treasury stock	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-
Total stockholders' equity	**71.3**	**171.3**	**152.1**	**133.1**	**115.2**	**115.2**	**93.0**	**63.8**	**233.5**	**493.8**	**493.8**	**456.1**	**415.4**	**562.8**	**510.3**	**510.3**	**605.5**	**300.2**	**428.1**	**61.9**
Total liabilities and stockholders' equity	**91.4**	**191.4**	**172.2**	**153.2**	**136.9**	**136.9**	**111.6**	**84.1**	**257.2**	**517.5**	**517.5**	**479.8**	**439.1**	**586.5**	**534.0**	**534.0**	**629.2**	**323.9**	**451.8**	**85.6**

Source: Jefferies estimates, Company reports

Exhibit 18 - MPLT Cash Flow Statement

Fiscal Year (Ending in December) ($ figures in MM, except per share)	FY23	FY24	1Q25 Mar-25	2Q25 Jun-25	3Q25E Sep-25	4Q25E Dec-25	FY25E	1Q26E Mar-26	2Q26E Jun-26	3Q26E Sep-26	4Q26E Dec-26	FY26E	FY27E	FY28E	FY29E	FY30E
Cash flows from operating activities:																
Net income	**(55.7)**	**(77.6)**	(22.3)	(29.8)	(29.4)	(37.0)	(118.6)	(38.9)	(42.2)	(48.2)	(54.2)	(183.5)	(277.3)	(315.1)	(368.4)	(379.2)
Adjustments to reconcile net income to net cash provided -																
Depreciation and amortization	0.6	(1.0)	(0.1)	0.2	0.3	0.3	0.8	0.3	0.3	0.3	0.3	1.3	1.3	1.3	1.3	1.3
Non-cash lease expense	0.5	0.7	0.2	0.2	0.2	0.2	0.9	0.2	0.2	0.2	0.2	0.9	0.9	0.9	0.9	0.9
Loss (gain) from equity method investment	(1.0)	1.0	-	-	-	-	-	-	-	-	-	-	-	-	-	-
Stock-based compensation	1.0	1.1	0.2	0.1	0.1	0.5	1.0	0.7	0.9	1.1	1.2	3.9	6.5	7.7	9.1	10.8
Loss from revaluation of preferred stock purchase rig	0.5	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-
Loss on disposal of property and equipment	-	0.1	-	-	-	-	-	-	-	-	-	-	-	-	-	-
Non-cash interest expense (income)	-	-	-	0.5	(0.5)	-	-	-	-	-	-	-	-	-	-	-
Change in deferred tax, other	-	-	-	-	(0.2)	-	(0.2)	-	-	-	-	-	-	-	-	-
Changes in operating assets and liabilities																
Increase in inventory	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-
(Increase) decrease in prepaid expenses	0.5	(2.3)	(0.2)	(6.7)	(3.9)	-	(10.8)	-	-	-	-	-	-	-	-	-
(Increase) decrease in receivables	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-
(Increase) decrease in other assets	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-
Deferred revenue	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-
Increase (decrease) in accounts payable	(0.4)	(1.3)	(1.1)	1.2	0.3	-	0.5	-	-	-	-	-	-	-	-	-
Accrued expenses	3.9	2.2	(2.1)	1.3	2.6	-	1.8	-	-	-	-	-	-	-	-	-
Operating lease liability	(0.3)	(0.7)	(0.2)	(0.2)	(0.2)	-	(0.6)	-	-	-	-	-	-	-	-	-
Increase (decrease) in other liabilities	(1.7)	(1.0)	-	(0.8)	(1.2)	-	(2.0)	-	-	-	-	-	-	-	-	-
Net cash provided by operating activities	**(52.0)**	**(78.8)**	**(25.6)**	**(34.0)**	**(31.8)**	**(36.0)**	**(127.3)**	**(37.7)**	**(40.7)**	**(46.6)**	**(52.4)**	**(177.4)**	**(268.6)**	**(305.2)**	**(357.1)**	**(366.2)**
Cash flows from investing activities:																
Maturities/sale of marketable securities	-	45.0	37.5	17.5	6.5	-	61.5	-	-	-	-	-	-	-	-	20.0
Purchases of marketable securities	-	(125.0)	-	-	(142.1)	-	(142.1)	-	-	-	-	-	-	-	-	-
Capital expenditures	(0.5)	(0.8)	-	(0.1)	(0.0)	-	(0.1)	-	-	-	-	-	-	-	-	-
Purchases of other securities	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-
Net cash used in investing activities	**(0.5)**	**(80.8)**	**37.5**	**17.4**	**(135.6)**	**-**	**(80.7)**	**-**	**-**	**-**	**-**	**-**	**-**	**-**	**-**	**20.0**
Cash flows from financing activities:																
Proceeds from issuance of common stock/series D, n	104.5	120.0	-	-	-	296.2	296.2	-	-	194.0	-	194.0	363.8	-	485.0	-
Proceeds from the exercise of stock options	0.0	0.3	0.0	0.0	0.1	-	0.1	-	-	-	-	-	-	-	-	-
Proceeds from issuance of convertible preferred stoc	-	-	-	-	198.9	-	198.9	-	-	-	-	-	-	-	-	-
Proceeds from debt borrowings	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-
Repayment of debt borrowings	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-
Other financing activities	-	(2.2)	(0.1)	(0.2)	(0.9)	-	(1.1)	-	-	-	-	-	-	-	-	-
Net cash provided by (used in) financing activities	**104.6**	**118.1**	**(0.0)**	**(0.2)**	**198.1**	**296.2**	**494.1**	**-**	**-**	**194.0**	**-**	**194.0**	**363.8**	**-**	**485.0**	**-**
Effect of foreign currency exchange rate changes on cash a	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-
Net increase in cash and cash equivalents	**52.1**	**(41.5)**	**11.9**	**(16.7)**	**30.7**	**260.2**	**286.1**	**(37.7)**	**(40.7)**	**147.4**	**(52.4)**	**16.6**	**95.1**	**(305.2)**	**127.9**	**(346.2)**
Cash and cash equivalents at beginning of period	27.9	80.0	38.5	50.4	33.7	64.4	38.5	324.6	286.9	246.2	393.6	324.6	341.2	436.3	131.1	258.9
Cash and cash equivalents at end of period	**80.0**	**38.5**	**50.4**	**33.7**	**64.4**	**324.6**	**324.6**	**286.9**	**246.2**	**393.6**	**341.2**	**341.2**	**436.3**	**131.1**	**258.9**	**(87.3)**

Source: Jefferies estimates, Company reports

Company Description

Maplight Therapeutics

MPLT is a clinical-stage biopharmaceutical company focused on addressing central nervous system disorders, including schizophrenia, alzheimer's disease psychosis (ADP), autism spectrum disorder (ASD), and Parkinson's disease.

Company Valuation/Risks

Maplight Therapeutics

Valuation is a probability of success (PoS)-weighted SOTP-based DCF analysis of pipeline products. Key risks include regulatory, clinical development, and commercialization.

Bristol-Myers Squibb Co

Valuation: Our DCF-based PT is supported by PE and PEG-relative valuations. Risks: R&D, LOEs, pricing, competition, legal/regulatory, M&A.

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(Article 3(1)e and Article 7 of MAR)

Recommendation Published	March 15, 2026 21:52 P.M.
Recommendation Distributed	March 15, 2026 21:52 P.M.

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The expected total return (price appreciation plus yield) for Buy rated securities with an average security price consistently below $10 is 20% or more within a 12-month period as these companies are typically more volatile than the overall stock market. For Hold rated securities with an average security price consistently

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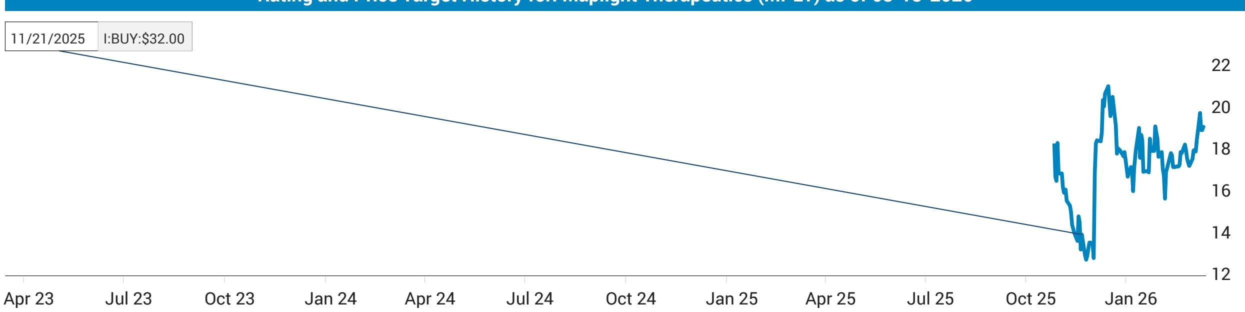
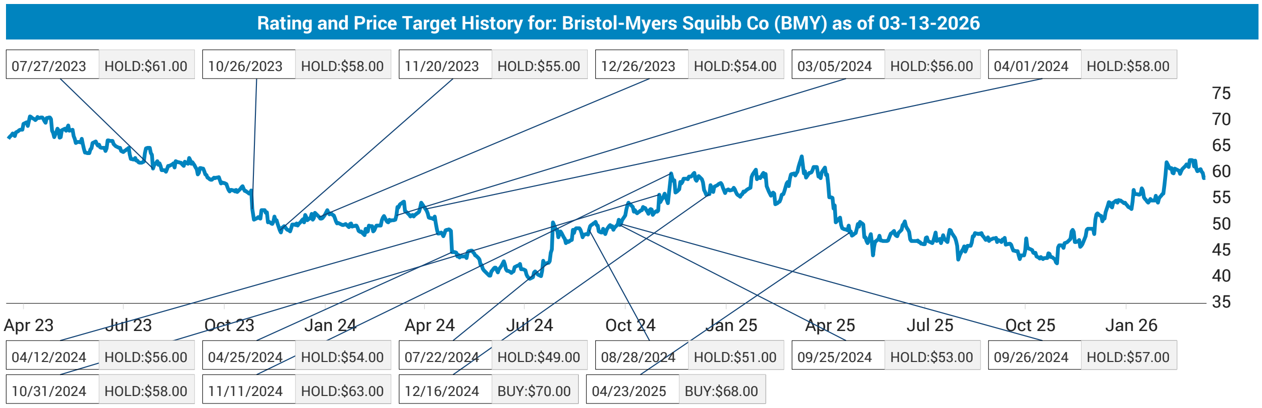
Other Companies Mentioned in This Report

- Bristol-Myers Squibb Co (BMY: $59.08, BUY)
- Maplight Therapeutics (MPLT: $19.07, BUY)

Rating and Price Target History for: Maplight Therapeutics (MPLT) as of 03-13-2026





Notes: Each box in the Rating and Price Target History chart above represents actions over the past three years in which an analyst initiated on a company, made a change to a rating or price target of a company or discontinued coverage of a company.

Legend:

I: Initiating Coverage

D: Dropped Coverage

B: Buy

H: Hold

UP: Underperform

Distribution of Ratings

			IB Serv./Past12 Mos.		JIL Mkt Serv./Past12 Mos.	
	Count	Percent	Count	Percent	Count	Percent
BUY	2163	61.64%	363	16.78%	108	4.99%
HOLD	1186	33.80%	98	8.26%	15	1.26%
UNDERPERFORM	160	4.56%	2	1.25%	2	1.25%

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